

Name : MS SWAPNA R

Registration No : MH002433933

Patient Episode : HM12000010158

Referred By : HOME CARE LAB SAMPLE
COLLECTION MVB

Receiving Date : 10 Dec 2025 07:28

Age : 46 Yr(s) Sex :Female

Lab No : 2501874382-2

Collection Date : 10 Dec 2025 07:28

Reporting Date : 11 Dec 2025 09:24

Clinical Laboratory Report

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Glycosylated Hemoglobin Specimen: EDTA HbA1c (Glycosylated Hemoglobin) Method: HPLC	5.9	%	[4.0-6.5]
As per American Diabetes Association(ADA) HbA1c in % Non diabetic adults >= 18years <5.7 Prediabetes (At Risk)5.7-6.4 Diagnosing Diabetes >= 6.5			
Estimated Average Glucose (eAG)	123	mg/dl	
Comments : HbA1c provides an index of average blood glucose levels over the past 8-12 weeks and is a much better indicator of long term glycemic control.			

-----END OF REPORT-----

Dr Rukmini
Consultant Biochemist

Authorized By : Dr Rukmini

Printed On : 07 Mar 2026 19:13

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Name : MS SWAPNA R **Age** : 46 Yr(s) Sex :Female
Registration No : MH002433933 **Lab No** : 2501879867-1
Patient Episode : HM12000010158 **Collection Date** : 10 Dec 2025 11:54
Referred By : HOME CARE LAB SAMPLE **Reporting Date** : 11 Dec 2025 08:51
Receiving Date : 10 Dec 2025 11:55

Clinical Laboratory Report

SEROLOGY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Hepatitis B Surface Antigen (HBSAG)			
Specimen-Serum			
Test Value:	4.66		
Cut Off Value:	1.00		
Test Method:	Chemiluminescent Microparticle Immunoassay.		
Test Result	REACTIVE		

Technical Note:

Hepatitis B virus infection is acquired through blood and blood products and maternal-neonatal transmission. HBSAg is the first serological marker of acute infection. HBV infection appearing 2-4 weeks after infection and becomes serologically undetected by ELISA within a few weeks to few months. Persistence HBSAg beyond 6 months indicates chronic HBV infection.

Method : Chemiluminescent Microparticle immunoassay (CMIA)
 (ABBOTT ARCHITECT KIT) REACTIVE

S/CO values are 4.66, 4.88 & 4.73 in the 1st and duplicate runs respectively
 S/CO values are 4.69, 4.66 & 4.38 in the 2nd and duplicate runs respectively

Reference Range for HBsAg test by Architect CMIA method :
 Non Reactive : S/CO value <1.00
 Reactive : S/CO $\geq$ 1.0.

Comment : The above mentioned test is a screening test. All HBsAg CMIA Reactive cases should be further evaluated by HBsAg neutralization assay and /HBV DNA PCR for confirmation.

Value informed to Dr. Naveen and Dr. Swathi by Arun at 18:25

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Receiving Date : 10 Dec 2025 11:55

Clinical Laboratory Report

SEROLOGY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Human Immunodeficiency Virus 1 & 2 (serology)			
Specimen-Serum			
Test Value:	0.13		
Cut Off Value:	1.00		
Test Method:	Chemiluminescent Microparticle Immunoassay		
Test Result:	Non-Reactive		
Comments:	Acquired Immunodeficiency syndrome (AIDS) is a disease caused by Human Immuno Deficiency Virus (HIV 1 and HIV 2). This test is a qualitative detection of antibodies to Human Immunodeficiency Virus types 1, including group M and O, and/or 2 (anti-HIV-1 and anti-HIV-2) and HIV p24 antigen. A non-reactive result does not preclude the possibility of infection with HIV. False positive results may be seen in autoimmune diseases, alcoholic hepatitis, primary biliary cirrhosis, leprosy and multiple pregnancies. The test may be non-reactive during the window period and end stage of the disease.		

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COLLECTION MVB

Reporting Date : 10 Dec 2025 15:51

Receiving Date : 10 Dec 2025 11:55

Clinical Laboratory Report

SEROLOGY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Hepatitis C virus Antibody (HCV) Specimen-Serum			
Test Value	0.13	S/CO	
Cut-off Value	:1.0		
Method:	Chemiluminescent Microparticle Immunoassay.		
Test Result:	Non-Reactive		

Technical Note:

Antibodies to hepatitis C virus appear after variable periods of time in different patients.
Other tests like recombinant immunoblot assay and PCR may be used for confirmation of results.

-----END OF REPORT-----

Dr Nazneen Fatima
Consultant Microbiologist

Authorized By : Dr Nazneen Fatima

Printed On : 07 Mar 2026 19:13

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Lab No : 2501879866-1

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Collection Date : 10 Dec 2025 11:55

Referred By : HOME CARE LAB SAMPLE
COLLECTION MVB

Reporting Date : 10 Dec 2025 15:38

Receiving Date : 10 Dec 2025 11:56

Clinical Laboratory Report

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
KIDNEY PROFILE			
Specimen: Serum			
GLUCOSE- Random	79	mg/dl	[70-140]
Method: GLUCOSE OXIDASE CONSUMPTION			
UREA	20.8	mg/dL	[17.0-43.0]
Method:Urease			
BUN, BLOOD UREA NITROGEN	9.7	mg/dl	[8.0-20.0]
Method: Calculated			
CREATININE, SERUM	0.86	mg/dl	[0.70-1.20]
Method: Enzymatic			
SODIUM, SERUM	130.81 #	mmol/L	[136.00-144.00]
POTASSIUM, SERUM	3.86	mmol/L	[3.60-5.10]
CHLORIDE, SERUM	102.0	mmol/L	[101.0-111.0]
SERUM BICARBONATE	20.0 #	mmol/L	[21.0-31.0]

Method: ISE Indirect

Technical Note

eGFR which is primarily based on Serum Creatinine is a derivation of CKD-EPI 2009 equation normalized to 1.73 sq.m BSA and is not applicable to individuals below 18 years. eGFR tends to be less accurate when Serum Creatinine estimation is indeterminate e.g. patients at extremes of muscle mass, on unusual diets etc. and samples with severe Hemolysis/ Icterus / Lipemia.

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Clinical Laboratory Report

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Free T4			Specimen: Serum
Serum FT4 (CHEMILUMINESCENCE)	1.05	ng/dL	[0.80-2.30]

-----END OF REPORT-----

Dr Rukmini
Consultant Biochemist

Authorized By : Dr Rukmini

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Receiving Date : 10 Dec 2025 07:28

Age : 46 Yr(s) Sex :Female

Lab No : 2501874383-1

Collection Date : 10 Dec 2025 07:28

Reporting Date : 10 Dec 2025 12:05

Clinical Laboratory Report

HAEMATOLOGY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
ERYTHROCYTE SEDIMENTATION RATE(Modified Westergren)			Specimen- EDTA whole Blood
ESR(Modified Westergren)	25.0 #	mm/1sthour	[0.0-20.0]

-----END OF REPORT-----

Dr VISHNUPRIYA V
Specialist - Pathology

Authorized By : Dr VISHNUPRIYA V

Printed On : 07 Mar 2026 19:13

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Registration No : MH002433933

Lab No : 2501874382-1

Patient Episode : HM12000010158

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COLLECTION MVB

Reporting Date : 10 Dec 2025 11:42

Receiving Date : 10 Dec 2025 07:28

Clinical Laboratory Report

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
THYROID PROFILE Serum			
THYROID PROFILE, Serum		Specimen: Serum	
T3 - Triiodothyronine (CHEMILUMINESCENCE)	0.940	ng/mL	[0.700-2.040]
T4 - Thyroxine (CHEMILUMINESCENCE)	8.630	µg/dL	[5.480-14.300]
Thyroid Stimulating Hormone (CHEMILUMINESCENCE)	6.410	#µIU/mL	[0.340-4.250]
1st Trimester:0.6 - 3.4	micIU/mL		
2nd Trimester:0.37 - 3.6	micIU/mL		
3rd Trimester:0.38 - 4.04	micIU/mL		

Note : TSH levels are subject to circadian variation, reaching peak levels between 22-4.a.m.and at a minimum between 6-10 pm.Factors such as change of seasons hormonal fluctuations,Ca or Fe supplements,high fibre diet,stress and illness affect TSH results.

* References ranges recommended by the American Thyroid Association

1) Thyroid. 2011 Oct;21(10):1081-125.PMID .21787128

2) <http://www.thyroid-info.com/articles/tsh-fluctuating.html>

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Clinical Laboratory Report

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
KIDNEY PROFILE			
Specimen: Serum			
GLUCOSE- Random	98	mg/dl	[70-140]
Method: GLUCOSE OXIDASE CONSUMPTION			
UREA	21.4	mg/dL	[17.0-43.0]
Method:Urease			
BUN, BLOOD UREA NITROGEN	10.0	mg/dl	[8.0-20.0]
Method: Calculated			
CREATININE, SERUM	0.85	mg/dl	[0.70-1.20]
Method: Enzymatic			
SODIUM, SERUM	135.94 #	mmol/L	[136.00-144.00]
POTASSIUM, SERUM	4.54	mmol/L	[3.60-5.10]
CHLORIDE, SERUM	107.0	mmol/L	[101.0-111.0]
SERUM BICARBONATE	19.0 #	mmol/L	[21.0-31.0]

Method: ISE Indirect

Technical Note

eGFR which is primarily based on Serum Creatinine is a derivation of CKD-EPI 2009 equation normalized to 1.73 sq.m BSA and is not applicable to individuals below 18 years. eGFR tends to be less accurate when Serum Creatinine estimation is indeterminate e.g. patients at extremes of muscle mass, on unusual diets etc. and samples with severe Hemolysis/ Icterus / Lipemia.

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Clinical Laboratory Report

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
GLUCOSE-Fasting Specimen: Plasma			
Plasma GLUCOSE-Fasting (GLUCOSE OXIDASE CONSUMPTION)	98	mg/dL	[70-100]
Serum LIVER FUNCTION TEST Specimen: Serum			
BILIRUBIN-TOTAL (JENDRASSIK GROF)**	0.42	mg/dL	[0.10-1.20]
BILIRUBIN - DIRECT (JENDRASSIK GROF)	0.07	mg/dL	[<0.2]
BILIRUBIN - INDIRECT (JENDRASSIK GROF)	0.35	mg/dL	[0.20-1.00]
SGOT/ AST (without P5P,UV)	27	U/L	[5-37]
SGPT/ ALT (without P5P,UV)	21	U/L	[10-50]
ALP (p-NPP)*	93	U/L	[39-100]
TOTAL PROTEIN (Biuret)	6.84 #	g/dL	[7.00-9.00]
SERUM ALBUMIN (Brocresol purple)	3.96	g/dL	[3.50-5.00]
SERUM GLOBULIN (Calculated)	2.88	g/dL	[1.80-3.40]
ALB/GLOB (A/G) Ratio(Calculated)	1.4		[1.1-1.8]
GGT (GCNA-IFCC)	29	U/L	[5-55]

Note:

**NEW BORN:Vary according to age (days), body wt & gestation of baby

*New born: 4 times the adult value

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BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
LIPID PROFILE			
Specimen: Serum			
TOTAL CHOLESTEROL (Direct Measure, polymer- polyanion)	216 #	mg/dL	[<200] <200 Moderate risk: 200 - 239 High risk: >240
TRIGLYCERIDES (ENZYMATIC WITH GLYCEROL BLANK)	166 #	mg/dL	[<150] Borderline high: 151-199 High: 200 - 499 Very high: >500
HDL - CHOLESTEROL (Direct Measure, polymer-polyanion)	50	mg/dL	[30-60]
VLDL - Cholesterol (Calculated)	33	mg/dL	[10-40]
LDL- CHOLESTEROL (Direct Measure, polymer-polyanion)	138 #	mg/dL	[<100] Near or Above optimal: 100-129 Borderline High: 130-159 High Risk: 160-189
T.CHOL/HDL.CHOL Ratio (Calculated)	4.3		<4.0 Optimal 4.0-5.0 Borderline >6 High Risk
LDL.CHOL/HDL.CHOL Ratio (Calculated Method)	2.8		<3 Optimal 3-4 Borderline >6 High Risk

Note:

Reference ranges based on ATP III Classifications.
Recommended to do fasting Lipid Profile after a minimum of 8 hours of overnight fasting.

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TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Specimen - Serum SERUM PHOSPHORUS (PHOSHOMOLYBATE)	3.43	mg/dL	[2.30-4.70]
Specimen : Serum URIC ACID Method: Uricase	5.60	mg/dl	[2.60-8.00]
Specimen - Serum CALCIUM - IONISED*	1.21	mmol/L	[1.15-1.32]
SERUM CALCIUM Method: ISE DIRECT	8.99	mg/dL	[8.60-10.00]

-----END OF REPORT-----

Dr Rukmini
Consultant Biochemist

Authorized By : Dr Rukmini

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Clinical Laboratory Report

HAEMATOLOGY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
COMPLETE BLOOD COUNT (Automated)		Specimen-EDTA Blood	
WBC COUNT(TC)(Coulter principle)	8.0	$\times 10^3$ cells/cumm	[4.0-11.0]
RBC Count (Electrical impedance)	4.07	million/cu.mm	[3.80-4.80]
Haemoglobin (Photometric method)	12.4	g/dl	[12.0-15.0]
Haematocrit (Calculated)	37.0	%	[36.0-46.0]
MCV (Calculated)	91.0	fl	[83.0-101.0]
MCH (Calculated)	30.4	pg	[27.0-32.0]
MCHC (Calculated)	33.4	g/dl	[31.5-34.5]
Platelet count	397	$\times 10^3$ cells/cumm	[150-450]
RDW (CV) (Calculated)	13.6	%	[11.6-14.0]
DIFFERENTIAL COUNT (VCS technology & Microscopy)			
Neutrophils	64.3	%	[40.0-70.0]
Lymphocytes	24.3	%	[20.0-45.0]
Monocytes	7.6	%	[2.0-13.0]
Eosinophils	3.5	%	[1.0-6.0]
Basophils	0.3	%	[0.0-2.0]
Absolute Neutrophilic Count	5.1	$\times 10^3$ cells/cumm	[2.0-7.0]
Absolute Lymphocyte Count	1.9	$\times 10^3$ cells/cumm	[1.0-3.0]
Absolute Monocyte Count	0.61	$\times 10^3$ cells/cumm	[0.20-1.00]
Absolute Eosinophil Count	0.28	$\times 10^3$ cells/cumm	[0.02-0.50]
Absolute Basophil Count	0.02	$\times 10^3$ cells/cumm	[0.02-0.10]

-----END OF REPORT-----

Vishnupriya

Dr VISHNUPRIYA V
Specialist - Pathology

Authorized By : Dr VISHNUPRIYA V

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